National Cancer Control Programme (NCCP) 00279 · clinician draft

Cisplatin + etoposide with radiotherapy

> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Days 2–3 · match actual drugs and route

Active medicines: Etoposide

Emetogenic risk: Route / schedule dependent

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- NCIS multiday guidance: give antiemetics appropriate to each day and for two days after. Continue the reconciled delayed cisplatin course; do not add another complete Day1 course.
- intravenous (IV) etoposide has low emetic risk. Oral etoposide requires the oral-route plan; split cisplatin means cisplatin is active on all three days. Pharmacy must issue one exact day-by-day prescription.
- No routine granulocyte colony-stimulating factor (white-cell growth support) (G-CSF), antibiotic, antiviral or Pneumocystis jirovecii pneumonia (PJP) prophylaxis is mandated by this NCCP document; additional prevention follows assessed risk/local policy.
- Assess eating/swallowing and refer for nutrition/pain support as needed.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00279 v8 · pNCCPpp1–2,5;NCISp5 · NCCP NCIS medical-oncology antiemetics v8

Sources checked 2026-11-02 · SACTCheck v0.84.30 · Regimen-level drafts; prescriber and oncology-pharmacy verification required